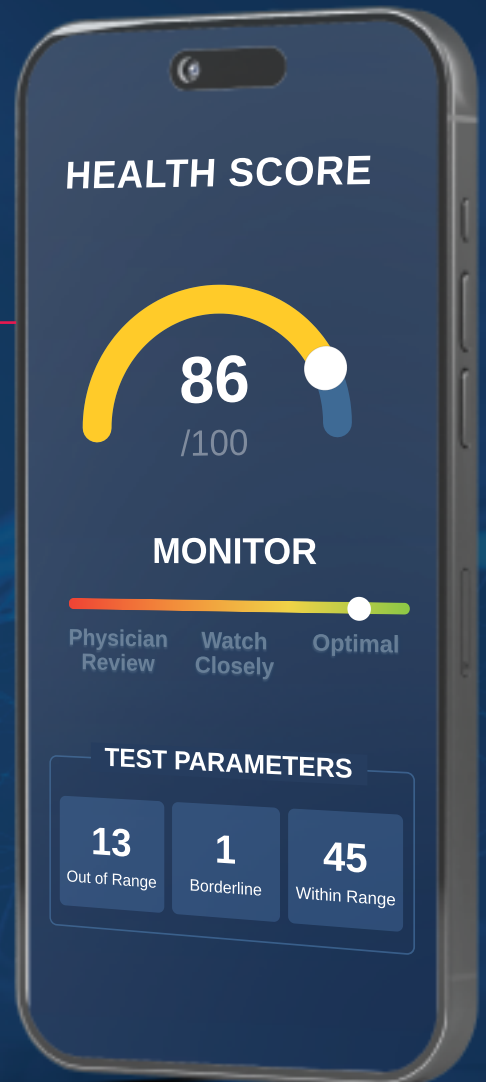


SMART HEALTH REPORT

An insightful Health Analytics Report
For Easier Understanding

Prepared For
**Mr Saksham
Upadhyay**
Male 26 yrs



Trusted by Millions Across India

Collected By:
Certified & Trained
Professionals

Processed At:
RRL NOIDA
NABL-Compliant Facility

Approved By:
Dr. Sakesh Kumar Agarwal
Dr. Walia
MD, Pathologists



Mr Saksham Upadhyay

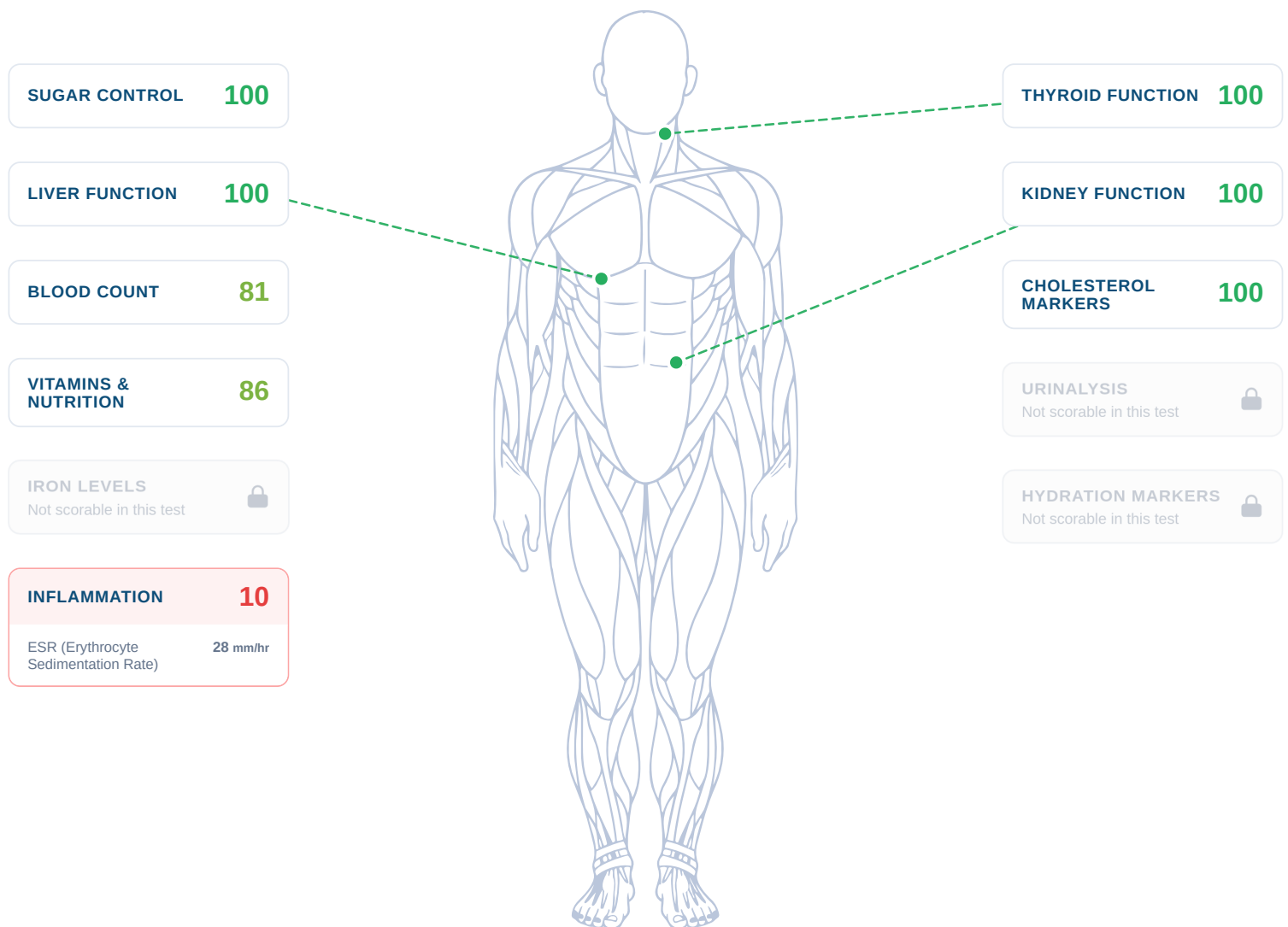
Gender: Male Age: 26 Yrs Patient ID: 17446833

Your Health Summary

Understand your health at a glance your personalised health summary is now available

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COMPREHENSIVE WELLNESS OVERVIEW



0-39

Physician Review

40-59

High Concern

60-69

Needs Action

70-79

Watch Closely

80-89

Monitor

90-100

Optimal

All domain scores are measured on a scale of 0 to 100, where a higher score indicates better health status



Mr Saksham Upadhyay

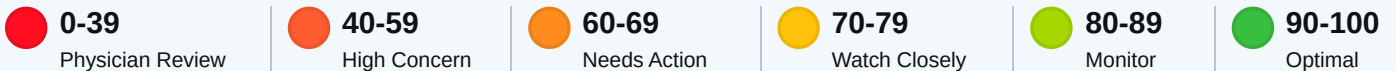
Gender: Male Age: 26 Yrs Patient ID: 17446833

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Health Score



HEALTH SCORE

MONITOR



TEST PARAMETERS



What is Health Score ?

Health Score is a comprehensive numerical indicator of an individual's overall health status. It converts complex medical data into a simple, easy-to-understand score that helps individuals quickly assess their current health and identify areas requiring attention.

Calculation Flow



BUN, P-LCC, eGFR (CKD-EPI)

These parameters are not part of your HealthScore calculation, as your HealthScore is calculated using selected parameters that reflect your overall wellness. Please consult your doctor if any of these values are flagged.



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

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DOCTOR CLINICAL OVERVIEW

BLOOD COUNT

Looks at blood cell counts and platelet parameters

Test Name	Result	Range
● Hemoglobin	11.5 g/dL	13 - 17
● TLC	4.8 $10^3/\mu\text{l}$	4 - 10
● Neutrophils	59.1 %	40 - 80
● Lymphocytes	31.1 %	20 - 40
● Monocytes	8.2 %	2 - 10
● Eosinophils	1.3 %	1 - 6
● Basophils	0.3 %	0 - 2
● Neutrophils.	2.84 $10^3/\mu\text{l}$	2 - 7
● Lymphocytes.	1.49 $10^3/\mu\text{l}$	1 - 3
● Monocytes.	0.39 $10^3/\mu\text{l}$	0.2 - 1.0
● Eosinophils.	0.06 $10^3/\mu\text{l}$	0.02 - 0.5
● Basophils.	0.01 $10^3/\mu\text{l}$	0.02 - 0.1
● Platelet Count	152 $10^3/\mu\text{l}$	150 - 410
● Mean Platelet Volume (MPV)	15.1 fL	9.3 - 12.1
● PDW	16.6 fL	8.3 - 25
● P-LCR	48.4 %	18 - 50
● P-LCC	47 $10^9/\text{L}$	44 - 140

● 3 Out of Range ● 0 Borderline ● 14 Within Range



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

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DOCTOR CLINICAL OVERVIEW

ANEMIA STUDIES

Evaluates red blood cell indices for anemia assessment

Test Name	Result	Range
● RBC Count	5.6 $10^6/\mu\text{l}$	4.5 - 5.5
● PCV	35.7 %	40 - 50
● MCV	63.7 fl	83 - 101
● MCH	20.5 pg	27 - 32
● MCHC	32.2 g/dL	31.5 - 34.5
● RDW (CV)	16.1 %	11.6 - 14.0
● RDW-SD	29.9 fl	35.1 - 43.9
● Mentzer Index	11.38 %	—

5 Out of Range 1 Borderline 2 Within Range

INFECTIOUS DISEASES

Markers indicating infection or immune response

Test Name	Result	Range
● PCT	0.2 %	0.17 - 0.32

0 Out of Range 0 Borderline 1 Within Range

INFLAMMATION

Inflammation markers including ESR, CRP, fibrinogen and cytokines

Test Name	Result	Range
● ESR - Erythrocyte Sedimentation Rate	28 mm/hr	0 - 10

1 Out of Range 0 Borderline 0 Within Range



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

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DOCTOR CLINICAL OVERVIEW

SUGAR CONTROL

Blood glucose and HbA1c for diabetes assessment

Test Name	Result	Range
● Glycosylated Hemoglobin (HbA1c)	5.1 %	0 - 5.7
● Estimated Average Glucose	99.67 mg/dl	—
● Glucose Fasting	77.9 mg/dL	70 - 100

0 Out of Range 0 Borderline 3 Within Range

LIVER FUNCTION

Liver enzymes, proteins and bilirubin levels

Test Name	Result	Range
● Bilirubin Total	1.5 mg/dL	0.2 - 1.2
● Bilirubin Direct	0.5 mg/dL	0 - 0.5
● Bilirubin Indirect	1 mg/dL	0.1 - 1
● SGOT/AST	32 U/L	11 - 34
● SGPT/ALT	37.4 U/L	0 - 45
● SGOT/SGPT Ratio	0.86 Ratio	0 - 0.99
● Alkaline Phosphatase	103.5 U/L	50 - 116
● Total Protein	7.6 g/dL	6.4 - 8.3
● Albumin	4.9 g/dL	3.5 - 5.2
● Globulin	2.7 g/dL	2.3 - 3.5
● Albumin :Globulin Ratio	1.81 Ratio	1.3 - 2.1
● Gamma Glutamyl Transferase (GGT)	15.8 U/L	0 - 55

1 Out of Range 0 Borderline 11 Within Range



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

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DOCTOR CLINICAL OVERVIEW

KIDNEY FUNCTION

Kidney filtration markers and waste products

Test Name	Result	Range
Blood Urea	34.4 mg/dL	19 - 44.1
Bun	16.07 mg/dL	6 - 20
Creatinine	0.7 mg/dL	0.6 - 1.2
eGFR (CKD-EPI)	130.31 ml/min/1.73 sq m	—
Uric Acid	7.3 mg/dL	3.7 - 7.7

Out of Range Borderline Within Range

CHOLESTEROL MARKERS

Cholesterol, cardiac enzymes and heart risk markers

Test Name	Result	Range
Total Cholesterol	152.5 mg/dL	0 - 239
Triglycerides	133.9 mg/dL	0 - 150
HDL Cholesterol	40 mg/dL	—
Non HDL Cholesterol	112.5 mg/dL	0 - 130
LDL Cholesterol	85.72 mg/dL	0 - 100
V.L.D.L Cholesterol	26.78 mg/dL	0 - 30
Chol/HDL Ratio	3.81 Ratio	0.0 - 5.0
HDL/ LDL Ratio	0.47 Ratio	0.3 - 0.4
LDL/HDL Ratio	2.14 Ratio	0 - 3.0

Out of Range Borderline Within Range



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

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DOCTOR CLINICAL OVERVIEW

VITAMINS & NUTRITION

Vitamins A, B, C, D, E, K and nutritional markers

Test Name	Result	Range
Vitamin - B12	< 148 pg/mL	187 - 883
Vitamin D 25 - Hydroxy	13.8 ng/mL	30 - 100

2 Out of Range 0 Borderline 0 Within Range

THYROID FUNCTION

Thyroid hormone balance

Test Name	Result	Range
Thyroid Stimulating Hormone (Ultrasensitive)	2.1 µIU/mL	0.35 - 4.94

0 Out of Range 0 Borderline 1 Within Range

**Mr Saksham Upadhyay**

Gender: Male Age: 26 Yrs Patient ID: 17446833

Your Health Summary

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PERSONALISED HEALTH SUMMARY

YOUR RESULTS INDICATE

Your inflammation marker ESR

Your inflammation marker ESR at 28 mm/hr is clearly raised. This marker climbs when something in the body is driving an inflammatory response, though on its own it does not say what. A doctor review this week is worth arranging.

Your blood count

Your blood count including MCV at 63.7 fl is below the healthy range. Haemoglobin carries oxygen from your lungs to every organ, so a level below range means your body has to work harder to move the same oxygen.

Your uric acid

Your uric acid at 7.3 mg/dL is slightly above the usual range. Uric acid is a normal waste product, but at higher levels it can crystallise in joints and add to the kidneys' workload over time.

HOW THIS MAY BE AFFECTING YOUR DAILY LIFE

Body aches and deep fatigue

Your inflammation markers, when raised, often shows up as body aches, joint stiffness, or lingering fatigue.

Reduced stamina and pale signs

Your blood count, at a moderate drop, you may notice pale inner eyelids or nail beds, or your heart beating noticeably harder during light activity. These are signs your blood is carrying less oxygen than it should — exactly what the hemoglobin number measures.

A quiet dietary signal

Your uric acid, at this level, joint discomfort is uncommon. Mildly raised uric acid is usually a silent signal about diet or hydration — most people notice no physical symptoms at this stage.



Mr Saksham Upadhyay

Gender: Male Age: 26 Yrs Patient ID: 17446833

Your Health Summary

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YOUR PERSONAL HEALTH ROADMAP

This Week

Do these first

1. Adding turmeric, leafy greens, and a daily handful of nuts is an easy anti-inflammatory start.
2. See your doctor this week to review your raised inflammation markers.
3. Notice whether minor colds, aches, or stiffness are taking longer than usual to settle.
4. A free Redcliffe report consultation can help you walk through your inflammation markers.

In the Next Month

Build momentum

1. Pairing turmeric with a pinch of black pepper, and protecting 7 hours of sleep, raises the effect — sleep and inflammation are closely linked.
2. Consider following up with your doctor next month to review the inflammation trend.
3. Over the month, notice whether morning stiffness and run-down days are becoming less frequent.
4. Planning an inflammation marker retest can help — only retest early if your doctor specifically advises it; meaningful changes typically take 8–12 weeks.

In 2–3 Months

Track your progress

1. Retesting an inflammation marker at the 2-3 month mark to confirm the trend is improving can help.
2. Review the 2-3 month inflammation results with your doctor.
3. A personalised dietician consultation is available within your first 30 days — speak to your Redcliffe health advisor or visit the app to check eligibility.
4. Making omega-3 foods — fish, or 1 tbsp of flaxseed — a few times a week, kept up over months, gives the most sustained benefit.



Smart Reports



AI Assistance



Report Consultation



Diet Plan



CARE by
Redcliffe Labs

Scan QR code



Patient NAME : Mr Saksham Upadhyay	Report STATUS : Final Report		
DOB/Age/Gender : 26 Y/Male	Barcode NO : 33185141		
Patient ID / UHID : 17446833/RCL17212730	Sample Type : Whole blood EDTA		
Referred BY : Self	Report Date : Jun 27, 2026, 02:15 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

Zyla0209 HS Home Collection

Complete Blood Count (CBC)

RBC Parameters			
Hemoglobin <i>Cyanide free spectrophotometry</i>	11.5 L*	g/dL	13.0 - 17.0
RBC Count <i>Electrical impedance</i>	5.6 H*	10 ⁶ /μl	4.5 - 5.5
PCV <i>Calculated</i>	35.7 L*	%	40 - 50
MCV <i>Calculated</i>	63.7 L*	fl	83 - 101
MCH <i>Calculated</i>	20.5 L*	pg	27 - 32
MCHC <i>Calculated</i>	32.2	g/dL	31.5 - 34.5
RDW (CV) <i>Calculated</i>	16.1 H*	%	11.6 - 14.0
RDW-SD <i>Calculated</i>	29.9 L*	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	4.8	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils <i>Flow-cytometry DHSS</i>	59.1	%	40 - 80
Lymphocytes <i>Flow-cytometry DHSS</i>	31.1	%	20 - 40
Monocytes <i>Flow-cytometry DHSS</i>	8.2	%	2 - 10
Eosinophils <i>Flow-cytometry DHSS</i>	1.3	%	1 - 6
Basophils <i>Flow-cytometry DHSS</i>	0.3	%	0 - 2
Absolute Leukocyte Counts			
Neutrophils. <i>Calculated</i>	2.84	10 ³ /μl	2 - 7
Lymphocytes. <i>Calculated</i>	1.49	10 ³ /μl	1 - 3
Monocytes. <i>Calculated</i>	0.39	10 ³ /μl	0.2 - 1.0
Eosinophils. <i>Calculated</i>	0.06	10 ³ /μl	0.02 - 0.5
Basophils.	0.01 L*	10 ³ /μl	0.02-0.1

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)

S. S. Agarwal

Dr. Sakesh Kumar Agarwal
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MBBS. DCP



Booking Centre :- Zyla ,
Processing Lab :- H-31, Sector 63, Noida-201301

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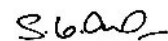
All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Saksham Upadhyay		Report STATUS : Final Report	
DOB/Age/Gender : 26 Y/Male		Barcode NO : 33185141	
Patient ID / UHID : 17446833/RCL17212730		Sample Type : Whole blood EDTA	
Referred BY : Self		Report Date : Jun 27, 2026, 02:15 PM.	
Sample Collected : Jun 27, 2026, 09:11 AM			

Test Description	Value(s)	Unit(s)	Reference Range
<i>Calculated</i>			
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	152	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) <i>Calculated</i>	15.1 H*	fL	9.3 - 12.1
PCT <i>Calculated</i>	0.2	%	0.17 - 0.32
PDW <i>Calculated</i>	16.6	fL	8.3 - 25.0
P-LCR <i>Calculated</i>	48.4	%	18 - 50
P-LCC <i>Calculated</i>	47	10 ⁹ /L	44 - 140
Mentzer Index <i>Calculated</i>	11.38	%	> 13

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Patient NAME	: Mr Saksham Upadhyay	Report STATUS	: Final Report
DOB/Age/Gender	: 26 Y/Male	Barcode NO	: 33185141
Patient ID / UHID	: 17446833/RCL17212730	Sample Type	: Whole blood EDTA
Referred BY	: Self	Report Date	: Jun 27, 2026, 02:54 PM.
Sample Collected	: Jun 27, 2026, 09:11 AM.		

Peripheral Smear General Blood Picture

RBCs	Predominantly microcytic hypochromic cells.
WBCs	Normal in number and distribution.
Platelets	Adequate in number on smear.
Hemoparasite	No Hemoparasite seen.
Impression	Microcytic hypochromic anemia.

Advise : 1) Kindly correlate clinically.

2) Serum iron profile.

A peripheral blood smear (also known as a general blood picture or blood film) is a laboratory test that involves examining a drop of blood under a microscope to evaluate the cells present. This test provides detailed information about the size, shape, and appearance of blood cells, which can help diagnose a variety of blood disorders and conditions.

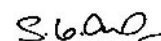
Clinical Use:

1. Diagnosing Anaemias: Helps determine the type and cause of anaemia.
2. Identifying Infections: Detects the presence of abnormal or immature white blood cells.
3. Blood Disorders: Diagnoses conditions like leukaemia, lymphoma, and other blood cancers.
4. Monitoring Treatment: Evaluates the effectiveness of treatment for blood disorders and infections.

Interpretation:

1. Normal Blood Smear: Shows normal size, shape, and colour of red blood cells, normal types and appearance of white blood cells, and normal number and appearance of platelets.
2. Abnormal Blood Smear: Can indicate a variety of conditions, depending on the specific abnormalities observed.

A peripheral blood smear is a valuable diagnostic tool that provides detailed information about the health and characteristics of blood cells, assisting in the diagnosis and management of a wide range of medical conditions.



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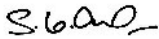
Patient NAME : Mr Saksham Upadhyay	Report STATUS : Final Report		
DOB/Age/Gender : 26 Y/Male	Barcode NO : 33185141		
Patient ID / UHID : 17446833/RCL17212730	Sample Type : Whole blood EDTA		
Referred BY : Self	Report Date : Jun 27, 2026, 03:25 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate MODIFIED WESTERGREN	28 H*	mm/hr	0 - 10
---	-------	-------	--------

Interpretation: ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc. Reference- Dacie and lewis practical hematology

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Patient NAME : Mr Saksham Upadhyay	Report STATUS : Final Report		
DOB/Age/Gender : 26 Y/Male	Barcode NO : 33185141		
Patient ID / UHID : 17446833/RCL17212730	Sample Type : Whole blood EDTA		
Referred BY : Self	Report Date : Jun 27, 2026, 03:38 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) HPLC	5.1	%	< 5.7
Estimated Average Glucose	99.67	mg/dl	Refer Table Below

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

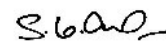
Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled. 2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413



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Patient NAME : Mr Saksham Upadhyay			
DOB/Age/Gender : 26 Y/Male		Report STATUS : Final Report	
Patient ID / UHID : 17446833/RCL17212730		Barcode NO : 32053783	
Referred BY : Self		Sample Type : FLUORIDE F	
Sample Collected : Jun 27, 2026, 09:11 AM		Report Date : Jun 27, 2026, 03:17 PM.	
Test Description	Value(s)	Unit(s)	Reference Range

Blood Sugar Fasting

Glucose Fasting Hexokinase	77.9	mg/dL	70 - 100
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Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association

Comment :

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note

- 1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
- 2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.



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Patient NAME : Mr Saksham Upadhyay			
DOB/Age/Gender : 26 Y/Male		Report STATUS : Final Report	
Patient ID / UHID : 17446833/RCL17212730		Barcode NO : 34140937	
Referred BY : Self		Sample Type : Serum	
Sample Collected : Jun 27, 2026, 09:11 AM		Report Date : Jun 27, 2026, 04:36 PM.	
Test Description	Value(s)	Unit(s)	Reference Range

Blood Urea Nitrogen (Bun)

Blood Urea Urease	34.4	mg/dL	19 - 44.1
Bun Calculated	16.07	mg/dL	6 - 20

Interpretation:

- The blood urea nitrogen (BUN) test measures the amount of nitrogen in your blood that comes from the waste product urea. Urea is formed in the liver when the body breaks down proteins and is eventually eliminated by the kidneys.
- Low BUN levels can be caused by a variety of factors. Liver disease is a primary cause, as urea is produced in the liver, and any dysfunction in this organ can lead to lower BUN levels. Malnutrition or a low protein diet can also contribute, as inadequate protein intake reduces urea production. Overhydration, resulting from excessive fluid intake, can dilute the urea concentration in the blood. Additionally, pregnancy can lower BUN levels due to increased fluid retention and changes in metabolism.
- High BUN levels can result from various conditions. Kidney dysfunction is a primary cause, as impaired kidney function can reduce urea elimination. Dehydration, which decreases fluid volume in the body, can also concentrate urea. A high protein diet can increase urea production due to excessive protein intake. Heart failure can reduce blood flow to the kidneys, impairing their function. Gastrointestinal bleeding contributes to high BUN levels by breaking down blood proteins in the digestive tract. Certain medications, such as corticosteroids and antibiotics, can elevate BUN levels. Additionally, shock or severe stress can affect BUN levels by reducing blood flow to the kidneys.



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Booking Centre :- Zyla, .
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Patient NAME : Mr Saksham Upadhyay	Report STATUS : Final Report		
DOB/Age/Gender : 26 Y/Male	Barcode NO : 34140937		
Patient ID / UHID : 17446833/RCL17212730	Sample Type : Serum		
Referred BY : Self	Report Date : Jun 27, 2026, 04:36 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

Creatinine, Serum

Creatinine <i>Kinetic alkaline picrate</i>	0.7	mg/dL	0.6 - 1.2
eGFR (CKD-EPI)	130.31	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15

Interpretation:
Creatinine estimation is done to assess kidney function. It is not dependent on dietary factors. Normal values are obtained in kidney diseases, except in advanced renal failure and therefore its estimation is more valuable if coupled with clearance.

"eGFR test is applicable for patients aged 18 years or more."

Uric Acid

Uric Acid <i>Uricase</i>	7.3	mg/dL	3.7 - 7.7
Interpretation: Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.			



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Patient NAME : Mr Saksham Upadhyay	Report STATUS : Final Report		
DOB/Age/Gender : 26 Y/Male	Barcode NO : 34140937		
Patient ID / UHID : 17446833/RCL17212730	Sample Type : Serum		
Referred BY : Self	Report Date : Jun 27, 2026, 04:36 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

Liver Function Test (LFT)

Bilirubin Total <i>Diazonium Salt</i>	1.5 H*	mg/dL	0.2 - 1.2
Bilirubin Direct <i>Diazo Reaction</i>	0.5	mg/dL	0.0 - 0.5
Bilirubin Indirect <i>Calculated</i>	1	mg/dL	0.1 - 1.0
SGOT/AST <i>Enzymatic [NADH (without P-5-P)]</i>	32	U/L	11 - 34
SGPT/ALT <i>Enzymatic [NADH (without P-5-P)]</i>	37.4	U/L	< 45
SGOT/SGPT Ratio <i>Calculated</i>	0.86	Ratio	<1.00
Alkaline Phosphatase <i>Para-nitrophenyl phosphate (p-NPP)</i>	103.5	U/L	50 – 116
Total Protein <i>Biuret</i>	7.6	g/dL	6.4 - 8.3
Albumin <i>Colorimetric BCG</i>	4.9	g/dL	3.5 - 5.2
Globulin <i>Calculated</i>	2.7	g/dL	2.3 - 3.5
Albumin :Globulin Ratio <i>Calculated</i>	1.81	Ratio	1.3 - 2.1
Gamma Glutamyl Transferase (GGT) <i>L-Gamma-Glutamyl-3-Carboxy-4-Nitroanalide</i>	15.8	U/L	< 55

Please correlate clinically.

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

- 1. AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
- 2. ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
- 3. Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

- 1. Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Test Description	Value(s)	Unit(s)	Reference Range
2. High protein (Hyperproteinemia): Often due to dehydration or increased protein production. 3. Low albumin: Caused by poor diet, kidney, or liver disease. 4. High albumin: Usually due to severe dehydration.			

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Patient ID / UHID : 17446833/RCL17212730	Sample Type : Serum		
Referred BY : Self	Report Date : Jun 27, 2026, 04:22 PM.		
Sample Collected : Jun 27, 2026, 09:11 AM			
Test Description	Value(s)	Unit(s)	Reference Range

Lipid Profile

Total Cholesterol <i>Enzymatic</i>	152.5	mg/dL	Desirable : < 200 Borderline high: 200 - 239 High risk: > 240
Triglycerides <i>Glycerol phosphate oxidase</i>	133.9	mg/dL	<150
HDL Cholesterol <i>Accelerator Selective Detergent</i>	40	mg/dL	> 40
Non HDL Cholesterol <i>Calculated</i>	112.5	mg/dL	<130
LDL Cholesterol <i>Calculated</i>	85.72	mg/dL	<100
V.L.D.L Cholesterol <i>Calculated</i>	26.78	mg/dL	< 30
Chol/HDL Ratio <i>Calculated</i>	3.81	Ratio	0.0 - 5.0
HDL/ LDL Ratio <i>Calculated</i>	0.47 H*	Ratio	0.3-0.4
LDL/HDL Ratio <i>Calculated</i>	2.14	Ratio	0.0- 3.0

Interpretation:

Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year,. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.

Causes of variation include

- Diet: Levels spike 5–10 times after meals. Unhealthy high saturated fat diets like non veg diet sweetened beverages , fried or processed foods , and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity , sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like : Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

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DOB/Age/Gender : 26 Y/Male

Patient ID / UHID : 17446833/RCL17212730

Referred BY : Self

Sample Collected : Jun 27, 2026, 09:11 AM

Report STATUS : Final Report

Barcode NO : 34140937

Sample Type : Serum

Report Date : Jun 27, 2026, 04:22 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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HDL Cholesterol			
Low		High	
<40		>=60	

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India.

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Test Description	Value(s)	Unit(s)	Reference Range
Current Vascular Pharmacology, 2022, 20, 134-155. - Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637–1646. - P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van &#39;t Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis, - Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825–32. DOI: 10.3238/arztebl.2019.0825			

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Referred BY : Self	Report Date : Jun 27, 2026, 06:21 PM.		
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Test Description	Value(s)	Unit(s)	Reference Range

Vitamin B12 / Cyanocobalamin

Vitamin - B12 CMIA	< 148 L*	pg/mL	187 - 883
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Interpretation: Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms. Causes of vitamin B12 deficiency include: Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia). Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia). 1. Out of 140 healthy Indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy CMIA	13.8 L*	ng/mL	Deficient <20 Insufficient 21 - 29 Sufficient 30 - 100
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Interpretation:
 25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

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Test Description	Value(s)	Unit(s)	Reference Range

TSH, Ultrasensitive

Thyroid Stimulating Hormone (Ultrasensitive) CMIA	2.1	μIU/mL	0.35 - 4.94
--	-----	--------	-------------

Interpretation:

Pregnancy	Reference ranges TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Note:

TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:

- Diagnose Hypothyroidism and Hyperthyroidism
- Monitor T4 replacement or T4 suppressive therapy
- Quantify TSH levels in the subnormal range

Increased Levels : Primary hypothyroidism, Subclinical hypothyroidis, TSH dependent Hyperthyroidism, Thyroid hormone resistance

Decreased Levels: Grace disease, Autonomous thyroid hormone secretion, TSH deficiency

References:

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568

*** End Of Report ***



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Patient Data

Sample ID: 33185141
 Patient ID:
 Name:
 Physician:
 Sex:
 DOB:

Analysis Data

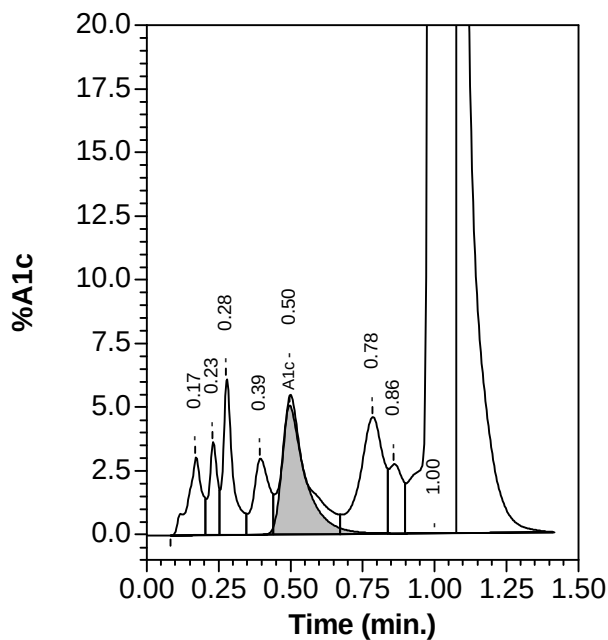
Analysis Performed: 06/27/2026 15:08:43
 Injection Number: 6875
 Run Number: 177
 Rack ID:
 Tube Number: 7
 Report Generated: 06/27/2026 15:27:27
 Operator ID:

Comments:

Peak Name	NGSP %	Area %	Retention Time (min)	Peak Area
A1a	---	1.5	0.168	28600
A1b	---	1.0	0.227	20099
F	---	2.1	0.275	40487
LA1c	---	1.7	0.394	33903
A1c	5.1	---	0.496	81794
P3	---	3.7	0.783	72994
P4	---	1.3	0.857	25075
Ao	---	84.6	1.000	1664518

Total Area: 1,967,471

HbA1c (NGSP) = 5.1 %



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13. The laboratory shall not be liable for any delays in performance or failure to perform its obligations arising out of or in connection with events beyond reasonable control, including but not limited to natural disasters, pandemics, acts of God, strikes, labor disputes, transportation disruptions, regulatory or governmental actions, technical failures, or any other unforeseen circumstances.
14. All disputes arising from laboratory services shall be subject to the jurisdiction of courts located in Delhi.



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